Youth HCD

Supporting GWSG Health Plan Expansion into Secondary Schools

Prepared by: YPS Planning, Service Design
With inputs from: DEA, SEH, CYPP & Outreach (Youth)

Context

- This presentation **summarises the key insights and gaps in the 13 to 16yo segment** and **outlines the proposed focus areas and approach** for the upcoming YPS Youth HCD study.
- To support GWSG expansion to youth segment, SD worked with YPS Divisions (CYPP, YPHS) and Integrated Outreach (Youth) to **stocktake existing HPB research and insights from the past five years**.
- Review shows strong coverage among older youths (17-25yo) but **limited insights on sec school (13-16yo)** & **transitional ages (12-13yo and 16-17yo)**.

Youth Segment Research (13 to 25 years old)

Mapping of Youth segment*

13-16 (early-mid adolescence)		17-19 (late adolescence)	20-25 (young adults)
Lower sec (13 – 14)	Upper sec (15 – 16)		

(Ongoing) Outreach Quarterly Huddle with **Youth Resource Panel**

(13-15yo) 2024 HCD, FGD
Youth Weight Management

(17-25yo) 2025 qualitative study:
Role of family & peers in influencing youth heath behaviours

(17-25yo) 2023 research survey:
Youth mindshare & social norms

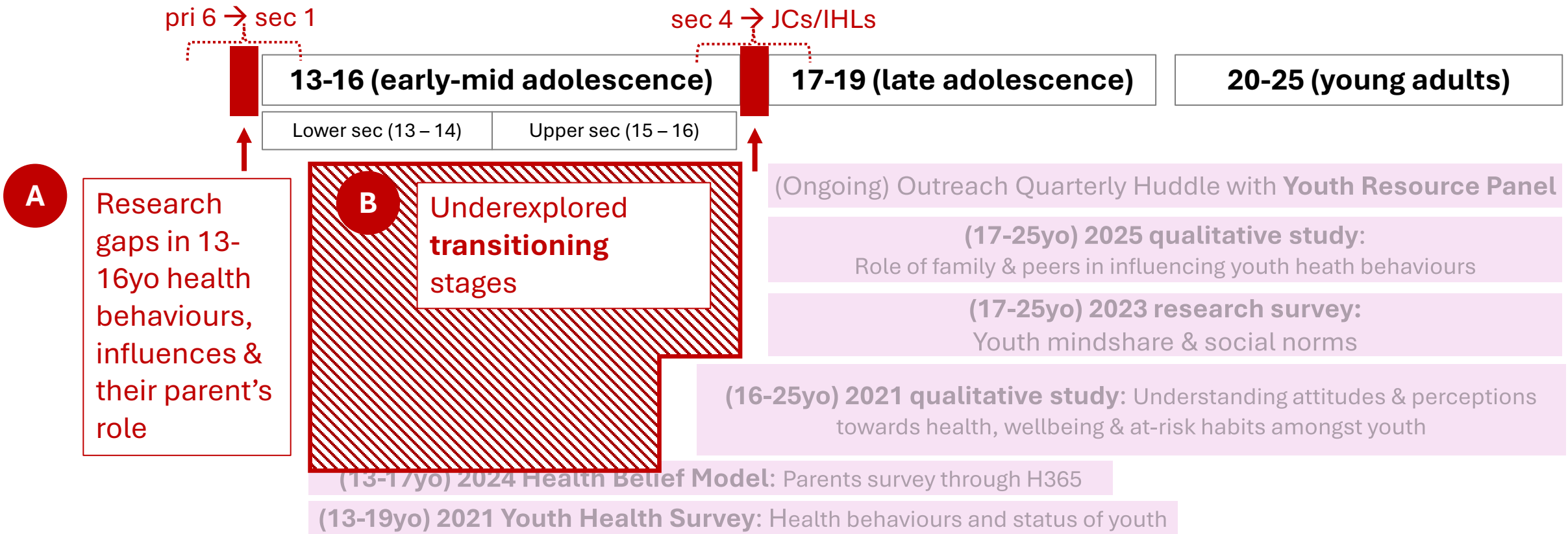
(16-25yo) 2021 qualitative study: Understanding attitudes & perceptions towards health, wellbeing & at-risk habits amongst youth

(13-17yo) 2024 Health Belief Model: Parents survey through H365

(13-19yo) 2021 Youth Health Survey: Health behaviours and status of youth

***Guided by:**
WHO defines adolescence as 10 – 19 years old and youth as 15 – 25 years old
MCCY defines Youth as 15 – 35 years old

We identified two key research gaps



Gaps

a. Limited insights into 13-16yo health behaviours, influences, and parental role.

b. Key transition periods (Pri 6 to Sec 1; Sec 4 to JC/IHLs) - underexplored despite being critical developmental stages where habits undergo significant shifts.

The upcoming HCD will address knowledge gaps in early-mid adolescence to inform tailored interventions and strengthen parental support during transitioning stages, and complementing Outreach's work with late adolescence and young adults.

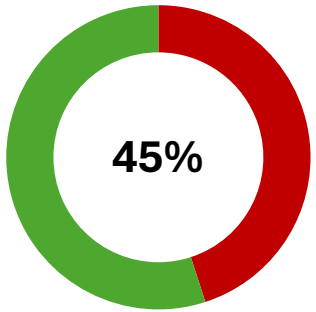
Our current understanding of Youth

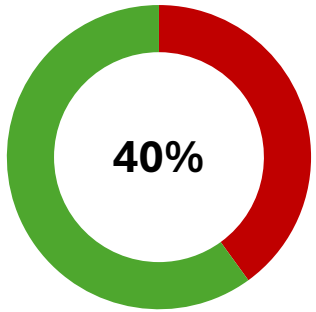
(focusing on early-mid adolescence)

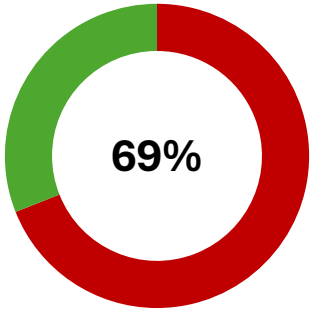
1. Status of Youth Health
2. Factors undermining youth from practicing healthy behaviours
3. Parents' role

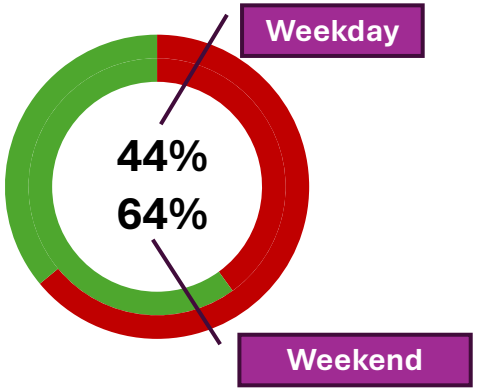
Youth in general are not performing well across the 5 Well, in particular sleep and screen

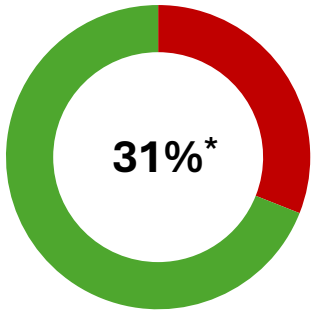
Prevalence of students not meeting guidelines for Exercise Well, Eat Well and Sleep Well:











Exercise Well

45% of Singaporean youths **not meeting minimum** 60-minutes daily activity requirements. Annex A

Eat Well

40.3% of youths **doubled intake** of sugar-sweetened beverages from 2015 to 2024. Annex A

40.6% of youths **did not meet recommended** fruit and vegetable consumption. Annex A

Sleep Well

69% of youths have **insufficient sleep**, up from 63.1% in 2015. Annex A

Singapore youths average only **7.12 hours – the shortest duration of sleep** compared to Japan, Hong Kong, India and Sri Lanka[1]

Learn Well

44% of youths spent **>3 hours** on devices during weekdays, rising to 64% on weekends. Annex B

Singaporean youths use their electronic devices for almost **8.5 hours on average daily**[2] **– Refer to Annex C for a day in a life around screen use and its interconnectedness**

Oral Health

Oral health worsens between 12 to 15y/o, and there is an increased prevalence of caries[3]

Considering GWSG 2.0 focus on mental health and digital space, we will dovetail efforts with parallel HCD led by MH team.

[1,2,3] Refer to Annex H
*Caries prevalence in 15 year olds
Datapoints extracted from YHS Survey includes JC group.

Youth have general health awareness but there are influences that undermines them from practicing healthy behaviours. The HCD study will seek to validate these influences.

Emerging themes derived from literature* and ground feedback:

01 Perceived Invincibility

Feel young and healthy now; perceived risk of future health problems is low

02 Competing Personal Goals

Physical appearances, academic pressure, career preparations, and social demands (e.g. peers) take precedence over long-term health

03 Knowledge Gaps

Awareness exists but specific knowledge lacking (e.g. minimum activity guidelines, proper oral care)

04 Translation Barriers

Inability to translate health knowledge into consistent daily behaviours & self-regulation. Esp. in areas such as

- Nutrition (shift from family-provided meals to eating out with poorer choices)
- Sleep (resisting parental routines but lack confidence establishing new routines).

Identified gaps to explore further

- How can we leverage youths' top concerns and align health with what matters most to them? Consider framing health from a non-health angle (to create a hook)
- How can we better support youths in translating health knowledge into action? What are the barriers and ecosystem levers (including peers) to empower them?

*Refer to Annex D, E and H

Parents role remains influential in shaping most youths' health but evolves as they grow[Annex F]. Research suggests it's the approach taken, with attuned, positive and supportive methods being most impactful

Evidence on parental involvement across 5 wells:

Exercise Well	Parental support was a **key influence on physical activity** in Singaporean youths[9]
Sleep Well	Parents setting bedtimes for youths appears to **slow the trend towards delaying bedtimes and decreasing sleep duration**[10]
Learn Well	Promoting digital literacy among parents could **minimise screen time** among youths[11]
Eat Well	Shared family meals are related to **nutritional health** in youths[11], and parent dietary intake is consistently correlated with youth dietary intake[12]
Oral Health	Parents' dental problems served as motivation for youths to **care more for their oral health**[13]

What matters – emerging themes

Sensitive attunement
- Noticing & responding without overly controlling

Positive engagement
- Encouragement, role modelling, shared activities

Monitoring & supervision
- Setting clear, flexible boundaries

Open communication
- Fostering safe, 2-way dialogues

Identified Gaps to Explore Further

- What parental support do youths value, and how can parents guide without controlling? How do youths convey their needs?
- To consider the specific nuances and challenges of parenting in the digital age

[9, 10, 11, 12, 13] Refer to Annex H

Youth continue to crave bonding with parents despite seeking independence. This presents an opportunity for parents to influence through meaningful connection

Youths' Perceptions on Parental Support

- Open to **receiving support for their sleep** from parents[10]
- Mixed views on exercising with parents, with some finding it motivating whilst others cited embarrassment & parents' inconsistent commitment[14]
- Credited parents for reinforcing oral self-care[15]
- Seek independence in decisions (screen time, routines, eating), yet still yearn for connection, esp. via shared activities

Parents' Perspectives on Supporting their Youths

- Tend to off-load responsibility to school/youth (particularly PA)[4]
- See transitions (e.g. Sec 1) as time for youths to take charge[4]
- Struggle with monitoring / enforcing healthy behaviours, especially screen use given youth's need for independence. (see Annex G)
- Want guidelines and tools for routines and modelling behaviours[16]
- Desire quality time through shared activities e.g. exercising or preparing healthy meals[4]

Bonding through shared activities remain the bridge – parents need support on how to balance guidance and youth's independence; youths need support on how to open up to their parents

[4, 10, 14, 15, 16] Refer to Annex H

In Summary…

1. The Youth HCD study will address knowledge gaps to deepen understanding of early-mid adolescence (13-16yo):

i. **Priorities, behaviours, influences to inform the approaches and design for sec school lifestyle interventions**

ii. **Parent-youth dynamics to generate actionable design insights for appropriate parental support as youth navigate transitions**

iii. **Key transition periods (Pri 6 to Sec 1; Sec 4 to JC/IHLs) where habits undergo significant shifts**

2. Key gaps to explore:

a) How can we leverage youths' top concerns and align health with what matters most to them?

b) How can we better support youths in translating health knowledge into action? What are the barriers and ecosystem levers (including peers) to empower them?

c) What parental support do youths value, and how can parents guide without controlling? What would make parents want to connect with their youths? (Vice versa, what would make youths open up to their parents and convey their needs?)

This is the last slide.

Annex

Annex A: Since 2014, Singaporean youths have consumed **more sugar-sweetened beverages**, and **insufficient wholegrains, fruits and vegetables**

Extracted from Youth Health Survey 2023

Summary of key findings

Domain area		Mainstream			ITE			Polytechnic		
		2015	2023	Trend status	2016	2023	Trend status	2014	2023	Trend status
Substance abuse	Prevalence of smoking	2.1%	0.5%		13.6%	3.6%		7%	3.3%	
	Prevalence of vaping	-	0.9%	N.A.	-	4.5%	N.A.	-	1.9%	N.A.
	Prevalence of binge drinking	7.2%	2.8%		17.6%	9.6%		20.8%	12.3%	
Mental health	Prevalence of poor mental health	17.9%	26.5%		22.5%	26.3%		17.2%	25.3%	
Sleep well	Prevalence of having sufficient sleep	36.9%	31%		44.2%	45.9%		57.5%	52%	
Exercise well	Prevalence of having sufficient physical activity	73.3%	55%		80.8%	55%		61.4%	63%	
Eat well	Prevalence of having sufficient fruit and vegetables	66.5%	59.4%		51.8%	38.1%		57.9%	41.5%	
	Prevalence of having sufficient wholegrains	60.6%	61.1%		51.1%	48.6%		50.8%	55.3%	
	Prevalence of having sugar sweetened beverages	25.8%	40.3%		36.8%	42.1%		22.3%	44.3%	
Health conditions	Prevalence of overweight / obesity	-	11%	N.A.	-	19%	N.A.	-	23%	N.A.
	Prevalence of impaired blood glucose	-	0.8%	N.A.	-	0.7%	N.A.	-	1.3%	N.A.
	Prevalence of elevated blood pressure	-	43%	N.A.	-	52%	N.A.	-	58%	N.A.
	Prevalence of hypertension	-	15%	N.A.	-	11%	N.A.	-	14%	N.A.
	Prevalence of abnormal blood lipids	-	12%	N.A.	-	17%	N.A.	-	18%	N.A.
	Prevalence of risk or ≥ 1 chronic condition	-	50%	N.A.	-	60%	N.A.	-	63%	N.A.

Sources: Student Health Survey 2014-2016, Youth Health Survey 2023

2

Annex B: **Screen Use** – 44% youths spent >3 hours on weekday and an even higher proportion (64%) of youths did so on weekends

Extracted from MDDI Digital Parenting Study 2025

Age Segment Overview – Secondary level children

As teens become more independent online, parents tend to step back from active monitoring and shift towards trust-based engagement(s). Communication efforts should focus on supporting teens directly to develop responsible digital behaviour, and help parents maintain open conversations and guidance.

Early Preschoolers (13%)
2-4 years old

Pre-primary learners (10%)
5-6 years old

Lower Primary Children (16%)
7-9 years old)

Upper Primary Children (15%)
10-12 years old

Lower Secondary Children (14%)
13-14 years old

Upper Secondary Children (32%)
15 -17 years old

Demographics Profile

Middle-Aged Parent Profile
More likely to be:
- Aged >40 for parents with children in lower sec (96% vs 73% in total sample)
- Aged >50 for parents with children in upper sec (53% vs 27% in total sample)

Learn well

High Digital Access and Screen Use
- More likely to own their own devices (97%)
- Children in this age group tended to spend the most time on devices – 44% of children spent 3 or more hours on weekday, and about 64% did so on weekends.

Parents' Attitudes and Concerns

Concern on Harmful Content and Interactions
- More likely than others to be concerned about child's exposure to **misinformation** (44%) and **interaction with strangers** (58%).

Maintained or Reduced Involvement
- While parents did express interest to use government resources that educates them on children's digital safety and well-being (65%), it was lower than parents with younger children.
- Those with children in lower secondary were more likely to **maintain** (i.e., not become more or less involved) their involvement in monitoring their child's digital activities (32%). Like parents of upper primary children, they also tended to monitor a wide range of their child's online activities.
- On the other hand, parents of children in upper secondary were more likely to be **less involved** (45%) as they tend to take the approach of trusting in their child to be responsible online and their desire to respect the child's privacy.

MINISTRY OF DIGITAL DEVELOPMENT AND INFORMATION

Annex C: A BMC Pediatrics study found that Singaporean youths have excessive phone use, creating a destructive cycle where poor sleep leads to fatigue, increasing screen dependency and further disrupting healthy behaviours.

A day in the life of youth[13] around screen use:

Weekdays | **Weekends**

→ Eliminates physical activity time and meaningful family or social interactions

Morning

Turn off alarms

Check the time, social media and messages

→ Later wake times reducing sleep quality leading to fatigue throughout the day.

To and In School

Used phones during commute to and fro

Also used phones before lessons, during recess, and in lessons (if permitted by the teacher)

After School

Used phones after or during lunch and/or dinner

Continued to use them during or after finishing homework

At Night

Used devices before bedtime

→ Disrupts sleep cycles, contributing to sleep deprivation and emotional stress

Weekends

Used devices throughout the whole day whenever possible

[13] Refer to Annex G

Annex D: Perceived vitality and prioritising future career and financial independence over long-term health prevents proactive healthy behaviours

Extracted from Asia Insights 2021

Without reframing health as relevant now, interventions risk being ignored.

GWSG efforts must tie health to immediate priorities (e.g. confidence, security, social acceptance) to shift behaviours.

OVERALL UNDERLYING BARRIER: I'M STILL YOUNG!

Youths believe their young age gives them invincibility, hence health and well being were not top-of-mind for most. Instead, youths considered future career and financial independence as more important now.

Th will continue to take a reactive and not roactive approach, unless their mindset was hanged to realise health issues are not age-gated.

NOTE! Priority on financial independence was stronger among youths from lower-income families / single-parent families, for a different reason:

- Youths from such families did not have a sense of safety / security with the family's finances and felt the need to be independent as soon as possible to protect themselves.
- Targeted outreach efforts to support such youths with financial stability so that they may pay more attention to themselves and their wellbeing.

Key Pain Points for Each Health Area:

Exercise

Most youths did not know the required amount of exercise for them (i.e. what constitute 150mins of MVPA?).
As such, some may think they had enough if they stood for work, or did housework, etc.

Nutrition

Barriers to better, more well-balanced diet lied in perception of healthy food being:
- Expensive
- Inconvenient / Not readily available
- Acquired taste

As such, not realistic to have it regularly / when with friends.

Mental Wellness

The only health area that youths saw as important, as many had experience.
However, lacking in:
- Openness to talk to others about it
- Ability to spot & support others with mental issues
- Use of healthy coping mechanisms

Substance Use

Most youths knew the harms of smoking, but harms of vaping were not as well known / accepted.
Youths did not know the threshold for binge drinking and harmful effects of binge drinking.

Sexual Health

Youths used protection more to prevent pregnancy, less for STD/STI.
Barriers to using protection:
- Spur-of-the-moment
- Judged for buying
- Expensive to buy

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Identified gap to explore further

- How might we leverage youths' top concerns to engage them more effectively in health behaviours that align with their values and aspirations?

Annex E: Youth are motivated by personal goals – looking good, progressing financially, and building relationships. These drivers can serve as natural entry points to promote healthy behaviours.

Extracted from Asia Insights 2025

ATTITUDE AND PRACTISE : Link how good health habits could achieve youth's personal goals

2

GOOD HEALTH HABITS :
Physically active, Good Nutrition habits, Good mental wellness, Substance control

GOALS

Physical appearance (e.g. beauty & grooming, physique)
Especially amongst the Secondary/ JC, ITE, Polytechnic students

Finance-related (e.g. saving/ earning money)
Especially amongst the University students and young working youths

Social aspect (e.g. bonding and relationships)
Especially amongst the Polytechnic and University students

NEEDS/ DESIRES

Look good → Be comparable to peers; Care about how others look at self; Find a job

To find desired employment/ career → To progress in career → To be better off financially

Build relationships → Get a girlfriend/ boyfriend; Improve relationship with parents

BENEFITS

Fear of losing out

Sense of self-esteem & confidence

Sense of security

Survival

Fulfil dreams and goals

Sense of belongingness

Health messages or programmes will be more effective if positioned as enablers to youths' personal goals, bridging the gap between long-term health goals and short-term gratification.

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Annex F: Parents remain influential in supporting youth health throughout the developmental, though their role evolves from early to late adolescence to meet changing needs

Adapted from Asia Insights 2025

High level of control over youth → Low level of control over youth

Life Stage	Early adolescence (10-13yo)	Mid adolescence (14-16yo)	Late adolescence (17-19yo)	Working Adults
Parent-Youth Dynamics	**Role of Parents:** Provide **structure** and **guidance** **Youth Needs:** Depend on parents for direction in forming healthy habits	**Role of Parents:** Shift towards being **coaches** and **boundary setters** **Youth Needs:** Seek **independence** while still requiring **parental guidance**	**Role of Parents:** Transition to **advisors** and **mentors** **Youth Needs:** Look for **emotional and practical support** but desire **greater autonomy**	**Role of Parents:** Act as a **support system** or **safety net** **Youth Needs:** Maitain **connection** while gaining full independence

Youths may test boundaries or seek distance, but parental influence continues to shape their decisions throughout.

Annex G: Parents seek guidelines and support due to barriers like limited knowledge, difficulty in enforcing rules with independent teens, and challenges on modeling healthy behaviours across the 5 Wells

Exercise Well

- Low awareness of physical activity guidelines[1]
- Reduced family physical activity time as parents off-loaded responsibility for physical activity to the school and their children[2]
- Despite parents' expectations that schools can meet youths' physical activity needs, this was only partly met unless youths had time or were motivated to take part in these activities[2]

Sleep Well

- Found it hard to set or monitor youths' sleep routines as parents were not aware that youths stay awake after bedtime[3]

Learn Well

- Difficulty enforcing device rules & restrictions; screen use limits are less effective due to independent access to device[4]
- Challenge to maintain device oversight, prevalent use in school and within peers[3]
- Struggled with role modelling good screen use behaviour[3]

Eat Well

- Interpretation of nutrition guidelines differs among family (e.g. how much in a serving)[5]
- Lack of nutritional knowledge to provide healthy meals[5]
- Little control over youths' choices outside[5]
- Limited ability for healthy eating food discussions due to busy schedule or own's poor eating habits[5]
- Choose unhealthy food for low cost[5]

Practise Good Oral Hygiene

- Parents with lower income and education levels exhibited less informed approaches to preserving their child's oral health (e.g. importance of regular dental check-ups, awareness of preventive dental treatments[6, 7]

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